

# Nathan's outreach kit, latest messaging

Prepared by Dallas Andrews • GTM Engineering • v2.4.1 • 2026-07-23 • 42 contacts

One section per contact: a defend-the-numbers box (current stars and the addressable math, all from public CMS 2026 data, none Intradiem-verified performance claims) then the five-touch sequence. Email 1 is the opener. Copy subject and body straight into Outlook.

# Devoted Health — Mel Chapman

SENIOR DIRECTOR OF MEMBER EXPERIENCE

To: melissa.chapman@devoted.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                 |
|---------------------------------------------------|------------------------------|----------------------|---------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>2.7 | CLINICAL STAR<br>4.2 | ADDRESSABLE (CS-SLICE)<br>\$31M |
| GROSS FORGONE QBP<br>\$44M                        | CONTRACTS<br>11              | MEMBERS<br>16,437    |                                 |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Devoted Health's last points sit

Hi Mel,

For plans sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data, the customer-service star across Devoted Health's contracts is sitting at 2.7 right now. That's roughly \$31M a year in addressable bonus across the 11 contracts, though you'll have a far sharper read on that than I will. The clinical side looks to be in decent shape by comparison, so the service measures are the lever your team can still pull this cycle.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Devoted's rating**

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Hi Mel,

One more angle on Devoted Health's gap across those 11 contracts. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. That makes the service side both most of what's holding the book back and the part of a cycle that pays back fastest.

You'll have a far sharper read than I will, but it's usually the piece teams underweight relative to how much it's actually worth.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Mel,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds: a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book sitting where Devoted Health's is, that service side is most of the gap to the bonus line.

Worth 15 minutes in the next couple weeks to walk the payer-level math?

Nathan Belfield

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**EMAIL 4 Subject: the Devoted Health measure breakdown**

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Hi Mel,

I can put together the measure-by-measure breakdown across Devoted Health's book: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest. That's the whole conversation, 15 minutes to walk it and you decide if there's anything there.

The complaints and customer-service measures price for this cycle only through December, so timing matters.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

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Hi Mel,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Devoted Health's book under the bonus line sits in the service measures, they're recoverable, and they price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure-by-measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# L.A. Care Health Plan — Jeff Ingram

DEPUTY CHIEF FINANCIAL OFFICER

To: jingram@lacare.org · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                          |                                     |                             |                                          |
|----------------------------------------------------------|-------------------------------------|-----------------------------|------------------------------------------|
| OVERALL BOOK<br><b>3.0 stars (a full star under 4.0)</b> | CUSTOMER-SERVICE STAR<br><b>3.1</b> | CLINICAL STAR<br><b>2.8</b> | ADDRESSABLE (CS-SLICE)<br><b>\$13.4M</b> |
| CONTRACTS<br><b>1</b>                                    | MEMBERS<br><b>30,796</b>            |                             |                                          |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the bonus dollars behind L.A. Care's 3.0

Hi Jeff,

There's roughly \$13.4M a year in quality-bonus dollars addressable through the service side of L.A. Care's Medicare Advantage book, and missing 4.0 by a fraction costs the same as missing it by a lot, it's a full cycle of bonus either way. The book is sitting at 3.0 right now, a full star under the line, though your read on the exact figure will be sharper than mine.

The points still in play this cycle sit in the customer-service and CAHPS measures, which come down to how the service operation runs day to day, so that's the part with room to move before the clinical side even enters the picture.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

If it helps, I can pressure-test that math against how you're framing the gap. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures carry L.A. Care's gap**

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Hi Jeff,

One more angle worth flagging. The gap doesn't sit evenly across L.A. Care's book, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting more heavily as the ratings evolve. That makes the service side the part with the fastest payback, since it's the piece a finance read usually underweights against the clinical side.

You'll have a far sharper read than I will on where that shows up hardest.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a single measure can cost**

---

Hi Jeff,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure has since retired, but the lesson holds: a single customer-service measure priced real money at scale.

The finance teams getting ahead of this aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time, which is what keeps L.A. Care's book from riding on any one number.

A pressure-test of how you're framing that exposure might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: L.A. Care's measure-by-measure breakdown**

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Hi Jeff,

I can put together a measure-by-measure breakdown for L.A. Care's book: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters more than it might seem.

Humana's freed up about two hours of capacity per agent every month working with us.

I can walk that breakdown with you in 15 minutes. Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

---

Hi Jeff,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding L.A. Care's book under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# CalOptima — Nancy H.

CHIEF FINANCIAL OFFICER

To: [nhuang@caloptima.org](mailto:nhuang@caloptima.org) · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                          |                                     |                             |                                         |
|----------------------------------------------------------|-------------------------------------|-----------------------------|-----------------------------------------|
| OVERALL BOOK<br><b>3.0 stars (a full star under 4.0)</b> | CUSTOMER-SERVICE STAR<br><b>2.7</b> | CLINICAL STAR<br><b>2.7</b> | ADDRESSABLE (CS-SLICE)<br><b>\$9.6M</b> |
| CONTRACTS<br><b>1</b>                                    | MEMBERS<br><b>18,849</b>            |                             |                                         |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: CalOptima's bonus dollars under 4.0

Hi Nancy,

There's roughly \$9.6M a year in quality-bonus dollars addressable across CalOptima's Medicare Advantage book once you get the contract over the 4.0 line, and missing by a fraction costs the same as missing by a lot, it's a full cycle of bonus either way. The contract is sitting at 3.0 right now, a full star under the line, though your read on the exact figure will be sharper than mine.

The points still in play this cycle are the operational ones, the customer-service measures, since those track how the service side runs day to day rather than anything clinical.

That's where our Dynamic Workforce Orchestration platform fits: it shifts work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: one more angle on the gap**

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Hi Nancy,

One more angle worth flagging on CalOptima's book. The gap doesn't sit evenly across the rating, it concentrates in the complaints and customer-service measures, the ones tied most directly to how the service operation runs day to day rather than anything clinical.

That also makes it the piece that pays back fastest once it moves, since those measures price for the last time this cycle before the weighting shifts further toward CAHPS.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what one bad call cost UHC**

---

Nancy,

Here's why the service side is worth pressing on now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, its own figure, under the prior rules. That specific measure has since retired, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book like CalOptima's, that's most of the gap to the bonus line.

Worth 15 minutes to walk through which measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 4 Subject: CalOptima's measure-by-measure breakdown**

---

Nancy,

I can put together a one-page breakdown for CalOptima's book: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters more than it looks like on paper.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks to walk through it? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: before the December window closes**

---

Hi Nancy,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding CalOptima's book under the bonus line sits in the service measures, and those price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before the window closes, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Medica — Krista Dusil

CHIEF FINANCIAL OFFICER

To: krista.dusil@medica.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.8 | CLINICAL STAR<br>3.6 | ADDRESSABLE (CS-SLICE)<br>\$23.7M |
| GROSS FORGONE QBP<br>\$61.2M                      | CONTRACTS<br>3               | MEMBERS<br>112,248   |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$23.7M riding on Medica's 4.0

Hi Krista,

There's roughly \$23.7M a year in quality-bonus dollars still addressable across Medica's Medicare Advantage contracts, and missing the 4.0 line by half a star costs the same as missing it by a lot, it's a full cycle of bonus either way. The book is sitting around 3.5 right now across three contracts, though your read on the exact figure will be sharper than mine.

The piece still in play this cycle is operational, the customer-service measures, which come down to how the service side runs day to day rather than anything clinical.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

If it helps, I can pressure-test that math against how your team is framing the gap. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures move first for Medica**

---

Hi Krista,

One more angle on the gap across Medica's contracts. It doesn't sit evenly across the rating, it concentrates in a handful of customer-service and complaints measures, the ones CMS keeps weighting heavier as the ratings move forward. That makes the service side the part most likely to pay back fastest, since it's operational rather than clinical, and the kind of thing finance teams often underweight when they model the path back to 4.0.

You'll have a far sharper read on your own book than I will.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Krista,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book sitting where Medica's is, that service side is most of the gap back to the bonus line.

A pressure-test of that math against your own framing might be worth doing. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: the Medica measure breakdown**

---

Hi Krista,

I can put the measure-by-measure breakdown for Medica's book on a single page: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

That's the whole conversation, 15 minutes to walk it and you decide if there's anything there. The complaints and customer-service measures price for this cycle only through December, so the timing matters.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

---

Hi Krista,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Medica's contracts under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure-level breakdown. Either way, appreciate you reading.

Nathan Belfield

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# CareFirst BlueCross BlueShield — Christopher Phillips

INTERIM CFO AND VP OF FINANCE | CORPORATE FP&A & CFO GOVERNMENT MARKETS

To: christopher.phillips@carefirst.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                  |
|---------------------------------------------------|------------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.9 | CLINICAL STAR<br>3.6 | ADDRESSABLE (CS-SLICE)<br>\$5.9M |
| GROSS FORGONE QBP<br>\$22.6M                      | CONTRACTS<br>1               | MEMBERS<br>46,358    |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$5.9M still in play at CareFirst

Hi Christopher,

There's roughly \$5.9M a year in quality-bonus dollars sitting in CareFirst's customer-service measures, part of an estimated \$22.6M across your Medicare Advantage book, and missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way. Your read on the exact figure will be sharper than mine.

The piece still movable this cycle is operational, how the service side runs day to day, since that's what the customer-service and CAHPS measures track.

That's where our Dynamic Workforce Orchestration platform fits: it shifts and reprioritizes work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures are actually movable here**

---

Hi Christopher,

One more angle on CareFirst's gap. It doesn't sit evenly across the rating, it concentrates in a handful of customer-service and complaints measures, the ones that price out for good this December before the weighting shifts further toward experience scores in future years.

That concentration is what makes the number worth a second look, since a narrow set of measures is carrying most of the addressable bonus.

You'll have a sharper read on which ones than I will. Worth 15 minutes to walk through which sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what one bad measure actually costs**

---

Christopher, here's why the service side is worth pressing on now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure is gone, but the lesson isn't: a single customer-service measure priced real money at scale.

The teams getting ahead of this aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book the size of CareFirst's, that service side is most of what's still movable this cycle.

A pressure-test of how you're framing that gap might be worth it. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: the measure breakdown for CareFirst**

---

Christopher, I can put together the measure-by-measure breakdown for CareFirst's book: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

That's the whole conversation, 15 minutes to walk it and you decide if there's anything worth acting on. The complaints and customer-service measures price for this cycle only through December, so timing is part of the math.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: leaving this here before December**

---

Hi Christopher,

I'll leave it here so I'm not crowding your inbox. The short version: the movable part of CareFirst's gap sits in the service measures, and those price for this cycle only through December.

If it's worth a look before then, just reply and I'll send the measure breakdown. Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

Either way, appreciate you reading.

Nathan Belfield

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# Devoted Health — Dan Quinn

VP, CHIEF ACTUARY

To: dquinn@devoted.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                 |
|---------------------------------------------------|------------------------------|----------------------|---------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>2.7 | CLINICAL STAR<br>4.2 | ADDRESSABLE (CS-SLICE)<br>\$31M |
| GROSS FORGONE QBP<br>\$44M                        | CONTRACTS<br>11              | MEMBERS<br>16,437    |                                 |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$31M riding on Devoted's line

Hi Dan,

There's an est. \$31M a year in quality-bonus dollars sitting in customer-service measures across Devoted Health's Medicare Advantage contracts, and missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way. Your book is sitting at 3.5 right now across 11 contracts, though your read on the exact figure will be sharper than mine.

The points still in play this cycle are the operational ones, since the customer-service measures come down to how the service side runs day to day, and those are scored only through December.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume swings. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

If it helps, I can pressure-test that math against how you're framing the gap. Worth 15 minutes in the next couple weeks?

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**EMAIL 2 Subject: one more angle on that gap**

---

Hi Dan,

One more angle on the math from my last note. Across Devoted's 11 contracts, the gap doesn't sit evenly, it concentrates in customer service and complaints, which happen to be the measures CMS is weighting harder through 2029. That's the part an operations team can actually move this cycle, and it's usually the piece finance models underweight because it looks operational rather than actuarial.

Worth 15 minutes to see which of those measures sit closest to a cut point across your book?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Dan,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure priced real money at scale.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. Across Devoted's book, that's the piece an operations team can still shift before the window closes.

Worth 15 minutes to walk the payer-level math against how you're modeling it?

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the payer-level measure breakdown**

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Dan,

I can put together the measure-by-measure breakdown across Devoted's 11 contracts: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk the breakdown? Either way, thanks for the work you're doing here.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: one last note before December**

---

Hi Dan,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Devoted's book under the bonus line sits in the service measures, and those price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before the window closes, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Medica HealthCare Plans — Shawn Larsen

VICE PRESIDENT QUALITY, STARS, AND RISK ADJUSTMENT

To: shawn.larsen@medica.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.8 | CLINICAL STAR<br>3.6 | ADDRESSABLE (CS-SLICE)<br>\$23.7M |
| GROSS FORGONE QBP<br>\$61.2M                      | CONTRACTS<br>3               | MEMBERS<br>112,248   |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Medica's last points sit

Hi Shawn,

For plans sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data your Medicare Advantage contracts sit right around 3.5, with the customer-service star at 3.8 and still the more movable side of the two. That's roughly \$23.7M a year in addressable bonus across your three contracts, though you'll have a far sharper read than I will.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Medica's rating**

---

Hi Shawn,

One more angle on Medica's gap. It doesn't sit evenly across the rating, it concentrates in a handful of customer-service and complaints measures, the ones CMS keeps weighting more heavily through 2029. That makes the service side most of what's holding all three contracts under the bonus line, and also the part that pays back fastest once it moves.

You'll have a far sharper read than I will, but it's usually the piece teams underweight relative to the clinical side.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a customer-service measure is worth**

---

Hi Shawn,

Here's why the service side is worth pushing on now, not later. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're moving several service and CAHPS measures at once in real time. For a book the size of Medica's, that's most of what's holding your contracts under the bonus line.

A look at which of those measures sit closest to a cut point might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the Medica measure breakdown**

---

Hi Shawn,

I can put the measure-by-measure breakdown for Medica's book on a single page: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest across all three contracts.

The complaints and customer-service measures price for this cycle only through December, so timing matters here. Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk through it?

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before Medica's December window**

---

Hi Shawn,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Medica's contracts under the bonus line sits in the service measures, they're recoverable, and they price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Clever Care Health Plan — LaTonya Augustine

DIRECTOR OF CAHPS AND STARS PROGRAMS

To: latonya.augustine@ccmapd.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                            |                      |                                   |
|---------------------------------------------------|----------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3 | CLINICAL STAR<br>3.8 | ADDRESSABLE (CS-SLICE)<br>\$21.2M |
| GROSS FORGONE QBP<br>\$24.5M                      | CONTRACTS<br>1             | MEMBERS<br>50,156    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Clever Care's last points sit

Hi LaTonya,

For a contract sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data, the customer-service star is sitting at 3.0 right now, which is behind the clinical side. That works out to roughly \$21.2M a year in addressable bonus tied to those service measures, out of about \$24.5M total, though you'll have a far sharper read than I will.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Clever Care's rating**

---

Hi LaTonya,

One more angle on Clever Care's gap. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. That makes the service side most of what's holding your book under the bonus line, and also the part that pays back fastest once it moves.

You'll have a far sharper read than I will on which measures are closest to turning.

Worth 15 minutes to walk through which of those sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi LaTonya,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to shift and reprioritize work across queues in real time so several service and CAHPS measures move at once. For Clever Care's book, that service side is most of what's keeping you under the bonus line.

A pressure-test of which measures sit closest to a cut point might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the Clever Care measure breakdown**

---

Hi LaTonya,

I can put the measure-by-measure breakdown for Clever Care's book on a single page: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

That's the whole conversation, 15 minutes to walk it and you decide if there's anything there. The complaints and customer-service measures price for this cycle only through December, so timing matters. Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before the December window**

---

Hi LaTonya,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Clever Care's book under the bonus line sits in the service measures, they're recoverable, and they price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Devoted Health — W. Bradley Yale

CHIEF FINANCIAL OFFICER

To: byale@devoted.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                 |
|---------------------------------------------------|------------------------------|----------------------|---------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>2.7 | CLINICAL STAR<br>4.2 | ADDRESSABLE (CS-SLICE)<br>\$31M |
| GROSS FORGONE QBP<br>\$44M                        | CONTRACTS<br>11              | MEMBERS<br>16,437    |                                 |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: Devoted Health's bonus dollars riding on 4.0

Hi Bradley,

There's roughly \$31M a year in quality-bonus dollars addressable across Devoted Health's Medicare Advantage contracts, and missing the 4.0 line by half a star costs the same as missing it by a lot, it's a full cycle of bonus either way. Your clinical star is already north of 4, so the piece holding the book under the line is the customer-service side, which is also the part still in play this cycle.

That's the operational lever, how the service operation runs day to day on complaints, appeals, and access, and it's what actually moves before December.

Our Dynamic Workforce Orchestration platform fits there: it reprioritizes work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where those dollars concentrate might be useful before the window closes. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures carry Devoted's gap**

---

Hi Bradley,

One more angle on the math from my last note. The gap across Devoted Health's contracts doesn't sit evenly, it concentrates in complaints, appeals, and access, the measures priced only through this December before the weighting shifts again.

That's the part finance usually underweights, since it reads as operational rather than actuarial, but it's the piece with a clock on it right now.

Worth 15 minutes to walk through which of those measures sit closest to a cut point across your 11 contracts?

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what one bad call cost UHC**

---

Hi Bradley,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has changed, but the lesson holds: a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. Across Devoted Health's 11 contracts, that's the lever still open before the window closes.

A pressure-test of where your service math is most exposed might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: Devoted Health's measure-by-measure breakdown**

---

Hi Bradley,

I can put together the measure-by-measure breakdown across Devoted Health's 11 contracts: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures only price for this cycle through December, so the timing matters more than usual this year.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk through where your movable points actually sit?

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before Devoted Health's December window**

---

Hi Bradley,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Devoted Health's contracts under the bonus line sits in the service measures, and those price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before the window closes, just reply and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# Clover Health — Peter Kuipers

CHIEF FINANCIAL OFFICER

To: peter.kuipers@cloverhealth.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.1 | CLINICAL STAR<br>3.5 | ADDRESSABLE (CS-SLICE)<br>\$49.1M |
| GROSS FORGONE QBP<br>\$74.6M                      | CONTRACTS<br>1               | MEMBERS<br>153,117   |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$49m riding on Clover's 4.0 line

Hi Peter,

There's an est. \$49.1M a year in quality-bonus dollars sitting inside Clover Health's customer-service measures right now, and missing the 4.0 line by half a star costs the same as missing it by a lot, it's a full cycle of bonus either way. Your book is sitting at 3.5 stars, so that gap is close enough that the service side alone could be what decides it, though your read on the exact math will be sharper than mine.

The points still in play this cycle come down to how the service operation runs day to day, complaints, appeals, customer service, the parts that move on their own timeline.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: one more angle on that gap**

---

Hi Peter,

One more angle on Clover Health's book. The bonus dollars at stake don't spread evenly across the rating, they concentrate in complaints, appeals, and customer-service handling, the measures tied directly to how the service operation runs day to day. That's also the part with the shortest payback, since it's operational rather than clinical, though you'll have a sharper read on where it sits than I will.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi Peter,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds: a single customer-service measure priced real money, and CMS is still weighting that side of the rating heavily.

The operators getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For Clover Health's book, that's still the part of the gap most within reach this cycle.

A pressure-test of how you're framing that math might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: Clover Health's measure-by-measure breakdown**

---

Hi Peter,

I can put together a measure-by-measure breakdown for Clover Health's book: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us, which is the kind of operational shift that ends up moving these measures.

Worth putting 15 minutes on the calendar in the next couple weeks to walk through it?

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: leaving this here before December**

---

Hi Peter,

I'll leave it here so I'm not crowding your inbox. The reframe still holds: missing the bonus line by half a star costs a full cycle either way, and the service side is the part of that gap still movable this cycle. Those measures price for 2026 only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have. If it's worth a look before the window closes, just reply here.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

# Lumeris — Ben Grabski

CHIEF FINANCIAL OFFICER

To: bgrabski@lumeris.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                          |                                     |                             |                                         |
|----------------------------------------------------------|-------------------------------------|-----------------------------|-----------------------------------------|
| OVERALL BOOK<br><b>3.0 stars (a full star under 4.0)</b> | CUSTOMER-SERVICE STAR<br><b>3.1</b> | CLINICAL STAR<br><b>3.7</b> | ADDRESSABLE (CS-SLICE)<br><b>\$8.1M</b> |
| CONTRACTS<br><b>1</b>                                    | MEMBERS<br><b>10,976</b>            |                             |                                         |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$8.1M sitting on Lumeris's service side

Hi Ben,

There's an est. \$8.1M a year in addressable bonus dollars on Lumeris's Medicare Advantage book right now, and missing the 4.0 line by half a star costs the same as missing it by a lot, it's a full cycle of bonus either way. The customer-service star is sitting at 3.1, so that's likely where the gap concentrates, though you'll have a far sharper read on the numbers than I will.

The piece still movable this cycle is operational, the customer-service and complaints measures that track how the service side runs day to day.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Lumeris's rating**

---

Hi Ben,

One more angle on the gap at Lumeris. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. That makes the service side both most of what's holding the book under the bonus line and the part that pays back fastest once it moves.

Across a book of about 11,000 members, that's not a small lever. You'll have a far sharper read than I will, but it's usually the piece finance teams underweight.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a failed measure once cost**

---

Hi Ben,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since been retired, but the lesson holds: a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book sitting where Lumeris is, that service side is most of the gap to the bonus line.

Worth 15 minutes in the next couple weeks to walk through which measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the Lumeris measure-by-measure breakdown**

---

Hi Ben,

I can put together a measure-by-measure breakdown for Lumeris: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

That's the whole conversation, 15 minutes to walk it and you decide if there's anything worth acting on. The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks? Either way, thanks for the work you're doing here.

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before the December window closes**

---

Hi Ben,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Lumeris under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# Clever Care Health Plan — Steven Haggard

DIRECTOR, MEMBER EXPERIENCE/OPERATIONS

To: steven.haggard@ccmapd.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                            |                      |                                   |
|---------------------------------------------------|----------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3 | CLINICAL STAR<br>3.8 | ADDRESSABLE (CS-SLICE)<br>\$21.2M |
| GROSS FORGONE QBP<br>\$24.5M                      | CONTRACTS<br>1             | MEMBERS<br>50,156    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Clever Care's last points sit

Hi Steven,

For plans sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data Clever Care's Medicare Advantage contract sits right around 3.5, with the customer-service star at 3 against a clinical star closer to 3.8. That gap points to roughly \$21.2M in addressable bonus sitting in the service measures, though you'll have a far sharper read than I will.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Clever Care's rating**

---

Hi Steven,

One more angle on Clever Care's gap. It doesn't sit evenly across the rating, it concentrates in a handful of customer-service and complaints measures, which are also the ones CMS keeps weighting heavier as Stars moves forward. So the service side isn't just part of the story, it's the part that pays back fastest once it moves.

You'll have a far sharper read on which measures are closest than I will, but it's usually the piece operations teams underweight.

Worth a look at which of those measures sit closest to a cut point?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi Steven,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book sitting where Clever Care's is, that service side is most of the gap left to close.

A pressure-test of which measures sit closest to a cut point might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the Clever Care measure breakdown**

---

Hi Steven,

I can put the measure-by-measure breakdown for Clever Care's book on a single page: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk it? Either way, thanks for the work you're doing here.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before the December window**

---

Hi Steven,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Clever Care's rating under the line sits in the service measures, and those are recoverable. The customer-service and complaints measures price for this cycle only through December, so the window matters.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown.

Nathan Belfield

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# Devoted Health — Julia Rosenbaum

ASSOCIATE VICE PRESIDENT, STARS PROGRAM

To: jrosenbaum@devoted.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                 |
|---------------------------------------------------|------------------------------|----------------------|---------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>2.7 | CLINICAL STAR<br>4.2 | ADDRESSABLE (CS-SLICE)<br>\$31M |
| GROSS FORGONE QBP<br>\$44M                        | CONTRACTS<br>11              | MEMBERS<br>16,437    |                                 |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Devoted Health's last points sit

Hi Julia,

For plans sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data your Medicare Advantage contracts sit right around 3.5, with the customer-service star well below the clinical side, which is holding at 4.2. That puts an est. \$31M a year in addressable bonus across your 11 contracts in the service measures alone, though you'll have a far sharper read than I will.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Devoted's rating**

---

Hi Julia,

One more angle on Devoted Health's gap. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. So the service side is most of what's holding the book under the line, which also makes it the part that pays back fastest once it moves.

You'll have a far sharper read than I will, but it's usually the piece teams underweight.

Worth 15 minutes to walk which of those measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what one service measure cost UHC**

---

Hi Julia,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure has since been retired, but the lesson holds: a single customer-service measure priced real money.

Teams getting ahead of this aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book like Devoted Health's, that's most of the movable gap sitting under the bonus line.

Worth 15 minutes in the next couple weeks to walk the payer-level math?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: Devoted Health's measure-by-measure breakdown**

---

Hi Julia,

I can put together a measure-by-measure breakdown for Devoted Health's book: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest across your 11 contracts.

The complaints and customer-service measures price for this cycle only through December, so the timing matters more than it looks.

Humana's freed up about two hours of capacity per agent every month working with us.

I can have that breakdown ready to walk through. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before the December window**

---

Hi Julia,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Devoted Health's book under the line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the service-measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Medica — Julee Smith

DIRECTOR, MEDICARE STARS

To: julee.smith@medica.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.8 | CLINICAL STAR<br>3.6 | ADDRESSABLE (CS-SLICE)<br>\$23.7M |
| GROSS FORGONE QBP<br>\$61.2M                      | CONTRACTS<br>3               | MEMBERS<br>112,248   |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Medica's last points sit

Hi Julee,

For plans sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data your Medicare Advantage contracts sit right around 3.5 across the book, with the customer-service star at 3.8 and the clinical side a bit behind that. About \$23.7M a year in addressable bonus across Medica's three contracts sits in those service measures, though you'll have a far sharper read than I will.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Medica's rating**

---

Hi Julee,

One more angle on Medica's gap across the three contracts. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier as Stars evolve. That makes the service side most of what's holding the book under the bonus line, which also makes it the part that pays back fastest once it moves.

You'll have a far sharper read than I will, but it's usually the piece teams underweight.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi Julee,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure has since been retired, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book like Medica's, sitting where it does across three contracts, that service side is most of the gap to the bonus line.

A pressure-test of that math against how you're framing the gap might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: Medica's measure-by-measure breakdown**

---

Julee, I can put together a measure-by-measure breakdown across Medica's three contracts: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk through it?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before the December window**

---

Hi Julee,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Medica's contracts under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Mass General Brigham — Niyum Gandhi

CHIEF FINANCIAL OFFICER AND TREASURER

To: ngandhi@massgeneralbrigham.org · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                  |
|---------------------------------------------------|------------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.7 | CLINICAL STAR<br>3.7 | ADDRESSABLE (CS-SLICE)<br>\$5.5M |
| GROSS FORGONE QBP<br>\$9.7M                       | CONTRACTS<br>2               | MEMBERS<br>17,823    |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the bonus math under Mass General Brigham's line

Hi Niyum,

There's an est. \$5.5M a year in quality-bonus dollars across Mass General Brigham's Medicare Advantage contracts sitting in measures tied to service operations, and missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way. The two contracts are at 3.5 right now, though your read on the exact figure will be sharper than mine.

The points still in play this cycle sit in the customer-service and CAHPS measures, which come down to how the service operation runs day to day.

That's where our Dynamic Workforce Orchestration platform fits: it shifts work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures carry Mass General Brigham's gap**

---

Niyum, one more angle on the math from last week. The service-star gap across your two Medicare Advantage contracts doesn't sit evenly, it concentrates in a handful of customer-service and complaints measures, the ones CMS keeps weighting through 2029. That's usually the piece finance models miss, since it recurs every year the contracts stay under the line rather than costing once. You'll have a far sharper read than I will on where yours actually sit. Worth 15 minutes to walk which measures across your 17,823 members sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi Niyum,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has changed, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book the size of Mass General Brigham's, that service side is most of what's still in your control this cycle.

Worth 15 minutes to walk the payer-level math across your contracts in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the Mass General Brigham measure breakdown**

---

Niyum,

I can put together a measure-by-measure breakdown for Mass General Brigham's contracts: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the needle fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters more than it looks. Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk the breakdown together?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: closing the loop before December**

---

Hi Niyum,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Mass General Brigham's contracts under the bonus line sits in the service side, it's recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before the window closes, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# Lumeris — Michael W. Gardner, EdD, MBA

CHIEF FINANCIAL OFFICER, LUMERIS BUSINESS UNITS

To: mgardner@lumeris.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                          |                                     |                             |                                         |
|----------------------------------------------------------|-------------------------------------|-----------------------------|-----------------------------------------|
| OVERALL BOOK<br><b>3.0 stars (a full star under 4.0)</b> | CUSTOMER-SERVICE STAR<br><b>3.1</b> | CLINICAL STAR<br><b>3.7</b> | ADDRESSABLE (CS-SLICE)<br><b>\$8.1M</b> |
| CONTRACTS<br><b>1</b>                                    | MEMBERS<br><b>10,976</b>            |                             |                                         |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$8.1M sitting under Lumeris's line

Hi Michael,

There's roughly \$8.1M a year in addressable bonus dollars riding on Lumeris's Medicare Advantage book clearing 4.0, and missing by a fraction costs the same as missing by a lot, it's a full cycle of bonus either way. The contract is sitting at 3.0 stars right now, a full star under the line, though your read on the exact figure will be sharper than mine.

The part still in play this cycle is operational, the customer-service measures, which come down to how the service side runs day to day rather than anything clinical.

That's where our Dynamic Workforce Orchestration platform fits: it shifts and reprioritizes work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: the piece of Lumeris's gap that recurs**

---

Hi Michael,

One more angle on Lumeris's number from last week. It's not spread evenly, it concentrates in the customer-service and complaints measures, which happen to be the ones CMS keeps weighting heavier through 2029. That makes the service side both the biggest lever and the fastest-paying part of any cycle you put against it.

You'll have a far sharper read on your own book than I will, but it's usually the piece finance models underweight because it doesn't look clinical on paper.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what one bad call cost UHC**

---

Hi Michael,

Here's why the service side is worth watching closely at Lumeris. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure can price real money into a rating.

Teams getting ahead of that aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book sitting where Lumeris's is, that's most of the gap to the bonus line.

A pressure-test of how you're framing that gap might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: Lumeris's measure-by-measure breakdown**

---

Michael, I can put together the measure-by-measure breakdown for Lumeris's Medicare Advantage book: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the service side fastest.

The complaints and customer-service measures only price for this cycle through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk through it? Either way, thanks for the work you're doing here.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: closing the loop before December**

---

Michael, I'll leave it here so I'm not crowding your inbox. The short version: Lumeris has roughly \$8.1M a year in addressable bonus dollars sitting in the service and CAHPS measures, and those measures price for this cycle only through December. Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have. If it's worth a look before the window closes, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

# Zing Health — Jennifer Hoenstine

VICE PRESIDENT, FINANCIAL PLANNING & ANALYSIS

To: jennifer.hoenstine@myzinghealth.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.0 stars (a full star under 4.0) | CUSTOMER-SERVICE STAR<br>2.3 | CLINICAL STAR<br>2.8 | ADDRESSABLE (CS-SLICE)<br>\$24.9M |
| CONTRACTS<br>2                                    | MEMBERS<br>36,021            |                      |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: Zing Health's bonus dollars under 4.0

Hi Jennifer,

There's roughly \$24.9M a year in addressable quality-bonus dollars across Zing Health's Medicare Advantage contracts, and missing the 4.0 line by a fraction costs the same as missing by a lot, it's a full cycle of bonus either way. The book is sitting at 3.0 stars right now, a full star under the line, though your read on the exact number will be sharper than mine.

The clinical side still has real distance to cover here, so the part that's actually movable this cycle is operational, the customer-service and CAHPS measures, since those track how the service side runs day to day.

That's where our Dynamic Workforce Orchestration platform fits: it shifts work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures carry Zing Health's gap**

---

Hi Jennifer,

One more angle on Zing Health's book. The gap doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. That makes the service side most of what's holding both contracts under the bonus line, and also the part that pays back fastest once it moves.

You'll have a far sharper read than I will on which measures are closest to turning.

Worth 15 minutes to walk through which ones sit closest to a cut point?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what one service measure was worth**

---

Hi Jennifer,

Here's why the service side is worth the attention now, beyond the math we've already walked. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That measure has since changed, but the lesson holds: a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book carrying Zing Health's gap, that's most of the ground still in play.

A pressure-test of how you're framing that gap might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the Zing Health measure breakdown**

---

Hi Jennifer,

I can put the measure-by-measure breakdown for Zing Health's contracts on a single page: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest. That's the whole conversation, 15 minutes to walk it and you decide if there's anything there.

The complaints and customer-service measures price for this cycle only through December, so timing matters.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before Zing Health's December window**

---

Hi Jennifer,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Zing Health's contracts under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our Dynamic Workforce Orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# Centene Corporation — Matt Hiltibidal

SENIOR DIRECTOR, FINANCE & PERFORMANCE MANAGEMENT

To: mhiltibidal@centene.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>2.8 | CLINICAL STAR<br>2.7 | ADDRESSABLE (CS-SLICE)<br>\$52.5M |
| GROSS FORGONE QBP<br>\$165.8M                     | CONTRACTS<br>21              | MEMBERS<br>60,651    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: \$52.5M sitting on Centene's service side

Hi Matt,

There's an est. \$52.5M a year in quality-bonus dollars across Centene's contracts riding on the service measures alone, and missing the 4.0 line by half a star costs the same as missing it by a lot, it's a full cycle of bonus either way. On the public data those 21 contracts are sitting at 3.5 right now, though your read on the exact number will be sharper than mine.

The piece still movable this cycle is operational, the customer-service and CAHPS measures, since those track how the service side runs day to day rather than the clinical side.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures carry Centene's gap**

---

Hi Matt,

One more angle on the service-star gap across Centene's book. It doesn't sit evenly, complaints and customer-service measures usually carry more of it than CAHPS does, and those are also the ones CMS keeps weighting into 2029. So the piece that pays back fastest is probably narrower than the full 21-contract picture suggests, though you'll have a far sharper read than I will on where it concentrates.

Worth 15 minutes to walk through which measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Matt,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure is gone, but the lesson isn't: a single customer-service measure priced real money, and CMS hasn't backed off that weighting.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. Across Centene's book, that's the piece still worth chasing this cycle.

A look at which measures sit closest to a cut point might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: the Centene measure breakdown**

---

Hi Matt,

I can put the measure-by-measure breakdown for Centene's book on a single page: which customer-service and CAHPS measures across the 21 contracts sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here. Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk it?

Nathan Belfield

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**EMAIL 5 Subject: before the December window closes**

---

Hi Matt,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Centene's book under the bonus line sits in the service measures, and those price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown.

Nathan Belfield

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# Clever Care Health Plan — Hiep Pham

CFO

To: hiep.pham@clevercarehealthplan.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                            |                      |                                   |
|---------------------------------------------------|----------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3 | CLINICAL STAR<br>3.8 | ADDRESSABLE (CS-SLICE)<br>\$21.2M |
| GROSS FORGONE QBP<br>\$24.5M                      | CONTRACTS<br>1             | MEMBERS<br>50,156    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$21m riding on Clever Care's service side

Hi Hiep,

There's an est. \$21.2M a year in quality-bonus dollars addressable across Clever Care's Medicare Advantage contracts tied to customer-service and CAHPS measures, and missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way. The book is sitting around 3.5 right now, though your read on the exact number will be sharper than mine.

The piece still in play this cycle is operational, how the service side runs day to day, since that's what the customer-service measures actually track.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures carry Clever Care's gap**

---

Hi Hiep,

One more angle on Clever Care's book. The service-star gap doesn't sit evenly across every measure, it concentrates in a handful of customer-service and complaints items, the ones that price out heaviest under the bonus formula.

That's usually the part finance teams underweight, since it's operational rather than clinical, and it's also the piece that pays back fastest once it moves. Across 50,000-plus members, even a small shift in those specific measures compounds.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hiep, here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since been retired, but the lesson holds: a single customer-service measure priced real money.

The operators getting ahead of this aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book sitting where Clever Care's is, that service side is most of what's left to close before the bonus line.

A pressure-test of that math against your own framing might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: the measure-by-measure breakdown for Clever Care**

---

Hiep, I can put together the measure-by-measure breakdown for Clever Care's book: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk the breakdown? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: before the December window closes**

---

Hiep, I'll leave it here so I'm not crowding your inbox.

The short version: the gap holding Clever Care's book under the bonus line sits in the service side, it's recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before the window closes, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Clover Health — Clay Thornton

INTERIM CFO, CLOVER HEALTH

To: clay.thornton@cloverhealth.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.1 | CLINICAL STAR<br>3.5 | ADDRESSABLE (CS-SLICE)<br>\$49.1M |
| GROSS FORGONE QBP<br>\$74.6M                      | CONTRACTS<br>1               | MEMBERS<br>153,117   |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: Clover's bonus dollars riding on 4.0

Hi Clay,

There's roughly \$49.1M a year in quality-bonus dollars still addressable across Clover Health's Medicare Advantage book, and missing the 4.0 line by a fraction costs the same as missing by a lot, it's a full cycle of bonus either way. The contract is sitting at 3.5 stars right now, half a star under the line, though your read on the exact figure will be sharper than mine.

The clinical star sits at 3.5 too, so the points still in play this cycle are the operational ones, the customer-service measures that come down to how the service operation runs day to day.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

If it helps, I can pressure-test that math against how your team is framing the gap. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures move Clover's rating**

---

Hi Clay,

One more angle on the gap we flagged. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier as it moves toward 2029. That makes the service side both the biggest lever and the fastest one to pay back, since every point recovered there compounds into future cycles too.

You'll have a far sharper read on this than I will, but it's usually the piece finance models underweight.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Clay,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure priced real money.

The operators getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time, so the gains compound instead of resetting each cycle.

For a book the size of Clover Health's, that service side is most of what's still recoverable this cycle.

Worth 15 minutes to walk through which measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 4 Subject: the Clover Health measure breakdown**

---

Clay, I can put the measure-by-measure breakdown for Clover Health's Medicare Advantage book on a single page. Which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk it? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: before the December window closes**

---

Hi Clay,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Clover Health's book under the bonus line sits in the service measures, they're recoverable, and they price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Community Health Plan of Washington — Stuart Battersby

CFO

To: stuart.battersby@chpw.org · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                          |                                   |                             |                                         |
|----------------------------------------------------------|-----------------------------------|-----------------------------|-----------------------------------------|
| OVERALL BOOK<br><b>3.0 stars (a full star under 4.0)</b> | CUSTOMER-SERVICE STAR<br><b>3</b> | CLINICAL STAR<br><b>3.1</b> | ADDRESSABLE (CS-SLICE)<br><b>\$4.9M</b> |
| CONTRACTS<br><b>1</b>                                    | MEMBERS<br><b>8,975</b>           |                             |                                         |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the bonus math under Community Health's 4.0 line

Hi Stuart,

There's roughly \$4.9M a year in addressable bonus dollars sitting on Community Health Plan of Washington's Medicare Advantage book, and missing the 4.0 line by a fraction costs the same as missing by a lot, it's a full cycle of bonus either way. On the public data the book is sitting at 3.0 stars right now, a full star under the line, though your read on the exact figure will be sharper than mine.

The part still in play this cycle is operational, the customer-service measures, which track how the service side runs day to day rather than the clinical side.

That's what our Dynamic Workforce Orchestration platform does: it shifts and reprioritizes work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures carry the gap**

---

Hi Stuart,

One more angle on Community Health Plan of Washington's book. The gap doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, which is also the part that pays back fastest once it moves. With 8,975 members on the affected contract, even a small shift in those measures compounds over more than one cycle.

You'll have a far sharper read than I will on where the room actually is.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Stuart,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure has changed, but the lesson holds: a single customer-service measure priced real money.

The teams getting ahead of this aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book sitting where Community Health Plan of Washington's does, that service side is most of the room you have to close the gap.

A pressure-test of that math against your own framing might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: the measure-level breakdown for your book**

---

Hi Stuart,

I can put together the measure-by-measure breakdown for Community Health Plan of Washington's book: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk through where your movable points actually sit?

Nathan Belfield

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**EMAIL 5 Subject: before the December window closes**

---

Hi Stuart,

I'll leave it here so I'm not crowding your inbox. The short version: roughly \$4.9M a year in addressable bonus dollars sits on Community Health Plan of Washington's book, the gap traces to the service side, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply and I'll send the measure breakdown. Either way, thanks for the work you're doing here.

Nathan Belfield

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# Cambia Health Solutions — Mike Rains

CHIEF FINANCIAL OFFICER

To: mike.rains@cambiahealth.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                  |
|---------------------------------------------------|------------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.2 | CLINICAL STAR<br>3.5 | ADDRESSABLE (CS-SLICE)<br>\$0.6M |
| GROSS FORGONE QBP<br>\$4.8M                       | CONTRACTS<br>3               | MEMBERS<br>51,411    |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: Cambia's bonus dollars riding on 4.0

Hi Mike,

There's about \$0.6M a year in quality-bonus dollars still addressable across Cambia's Medicare Advantage contracts, tied to the customer-service measures an operations team can still move this cycle. Missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way, though your read on the exact figure will be sharper than mine.

The clinical side sits close by too, but the service measures are the part that comes down to how the operation runs day to day, and that's what's still in play before the window closes in December.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the window closes. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Cambia's rating**

---

Hi Mike,

One more angle on the gap across Cambia's contracts. It doesn't sit evenly, it concentrates in the customer-service and complaints measures, the ones that price out for the last time this measurement year before CMS shifts weight toward the CAHPS experience side.

That makes the service side the piece that's both movable now and worth more the longer it stays fixed. You'll have a far sharper read on which measures are closest than I will.

Worth 15 minutes to walk through which ones sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what one bad call cost UHC**

---

Hi Mike,

Here's why the service side is worth pressing on now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That measure has since changed, but the lesson holds: a single customer-service measure priced real money at scale.

Teams getting ahead of it now aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time, which is where Cambia's gap sits too.

A look at which of those measures sit closest to a cut point could be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the Cambia measure breakdown**

---

Hi Mike,

I can put the measure-by-measure breakdown for Cambia's contracts on a single page: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters. Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks to walk it? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: leaving this before December**

---

Hi Mike,

I'll leave it here so I'm not crowding your inbox. The short version: the addressable dollars sitting in Cambia's service measures price for this cycle only through December, and that's the part an operations team can still move.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before the window closes, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Blue Shield of California — Kathryn Qin

INTERIM CHIEF ACTUARY

To: kathryn.qin@blueshieldca.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.6 | CLINICAL STAR<br>3.2 | ADDRESSABLE (CS-SLICE)<br>\$11.5M |
| GROSS FORGONE QBP<br>\$33.3M                      | CONTRACTS<br>3               | MEMBERS<br>45,627    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: bonus dollars riding on 4.0

Hi Kathryn,

There's roughly \$11.5M a year in quality-bonus dollars riding on whether Blue Shield of California's Medicare Advantage book clears 4.0, and missing by half a star costs the same as missing by a lot, it's a full cycle of bonus either way. The three contracts are sitting at 3.5 right now, though your read on the exact figure will be sharper than mine.

The points still in play this cycle sit in the customer-service measures, which come down to how the service operation runs day to day, and those get scored for the last time this December.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

If it helps, I can pressure-test that math against how you're framing the gap. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: one more angle on the gap**

---

Hi Kathryn,

One more angle on the math from my last note. Across the three contracts, the addressable dollars aren't spread evenly, they concentrate in a handful of customer-service and complaints measures, which happen to be the ones an operations team can still influence before December.

That's usually the piece finance models miss, since it reads as an ops question but it's really a bonus-payment question.

Worth a short call to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi Kathryn,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, its own figure, under the prior rules. That measure has since changed, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of this aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. Across Blue Shield's book, that's the piece sitting closest to the bonus line.

Worth 15 minutes to walk the payer-level math against how you're framing it?

Nathan Belfield

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**EMAIL 4 Subject: the measure-by-measure breakdown for you**

---

Kathryn, I can put together the measure-by-measure breakdown across Blue Shield's three contracts: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

That's the whole conversation, 15 minutes to walk it and you decide if there's anything there. Those measures price for this cycle only through December, so timing matters.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before the December window closes**

---

Hi Kathryn,

I'll leave it here so I'm not crowding your inbox. The short version: the bonus dollars still in play across Blue Shield's book sit in the customer-service measures, and those price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# IEHP — Carol Chio, FSA, MAAA

VICE PRESIDENT, ACTUARIAL SERVICES

To: chio-c@iehp.org · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.0 stars (a full star under 4.0) | CUSTOMER-SERVICE STAR<br>3.1 | CLINICAL STAR<br>2.5 | ADDRESSABLE (CS-SLICE)<br>\$14.8M |
| CONTRACTS<br>1                                    | MEMBERS<br>38,439            |                      |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the bonus dollars sitting under 4.0

Hi Carol,

There's roughly \$14.8M a year in addressable quality-bonus dollars sitting under the 4.0 line for IEHP's Medicare Advantage book, and missing by a fraction still costs a full cycle of bonus, same as missing by a lot. The book is sitting at 3.0 stars right now, a full star under the line, with no contract close enough to clear it in one cycle, so this reads as a longer-horizon push rather than a same-cycle fix, though your read on the exact math will be sharper than mine.

The piece still in play is the customer-service side, since those measures come down to how the service operation runs day to day.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures carry IEHP's gap**

---

Hi Carol,

One more angle on IEHP's book. The gap doesn't sit evenly across the rating, it concentrates in the complaints and customer-service measures, the ones CMS keeps weighting heavier through 2029. That makes the service side both the biggest piece of the gap and the part that pays back fastest once it moves.

Across roughly 38,000 members, that's not a small slice to leave sitting still.

You'll have a far sharper read than I will, but it's usually the piece actuarial models underweight. Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Carol,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, its own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book sitting where IEHP's is, that service side is most of the gap to the bonus line.

A pressure-test of that math against your own framing might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: IEHP's measure-by-measure breakdown**

---

Carol, I can put together a measure-by-measure breakdown for IEHP's book: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

Those measures price for this cycle only through December, so timing matters more than it looks.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk through the breakdown? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: before the December window closes**

---

Carol, I'll leave it here so I'm not crowding your inbox.

The short version: the gap holding IEHP's book under the bonus line sits in the customer-service measures, they're recoverable, and they price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# NYC Health + Hospitals — Zhishen Y.

CHIEF FINANCIAL OFFICER – LINCOLN MEDICAL CENTER

To: yangz7@nychhc.org · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                  |
|---------------------------------------------------|------------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.2 | CLINICAL STAR<br>3.8 | ADDRESSABLE (CS-SLICE)<br>\$5.4M |
| GROSS FORGONE QBP<br>\$6.6M                       | CONTRACTS<br>1               | MEMBERS<br>13,517    |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the dollars riding on NYC H+H's 4.0

Hi Zhishen,

There's roughly \$5.4M a year in quality-bonus dollars addressable across NYC Health + Hospitals' Medicare Advantage contracts, and missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way. The book is sitting around 3.5 right now, though your read on the exact figure will be sharper than mine.

The points still in play this cycle sit in the customer-service measures, which come down to how the service operation runs day to day rather than anything clinical.

That's where our Dynamic Workforce Orchestration platform fits: it shifts and reprioritizes work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures move NYC H+H's rating**

---

Hi Zhishen,

One more angle on the gap across your Medicare Advantage book. It doesn't sit evenly across the rating, it concentrates in a handful of customer-service and complaints measures, the kind an operations team can still move before the year closes out. That usually means the fix is narrower than it first looks, which is good news for where effort goes.

You'll have a far sharper read than I will on which ones are closest.

Worth 15 minutes to walk through which measures sit nearest a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Zhishen,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds: a single customer-service measure priced real money at that scale.

The teams getting ahead of this aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time, which is exactly the part of NYC Health + Hospitals' gap that's still movable this cycle.

Worth 15 minutes to walk through where your book's payer-level math actually lands?

Nathan Belfield

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**EMAIL 4 Subject: the measure breakdown for your book**

---

Zhishen, I can put a measure-by-measure breakdown together for NYC Health + Hospitals: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

That's the whole conversation, 15 minutes to walk it and you decide if there's anything there. The customer-service and complaints measures price for this cycle only through December, so timing matters more than usual right now.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

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**EMAIL 5 Subject: before the December window closes**

---

Zhishen, I'll leave it here so I'm not crowding your inbox. The short version: the gap holding NYC Health + Hospitals' book under the bonus line sits in the service measures, they're recoverable, and they price for this cycle only through December. Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have. If it's worth a look before the window closes, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Baystate Health — Laurie Martin

CHIEF FINANCIAL OFFICER

To: laurie.martin@baystatehealth.org · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                            |                      |                                   |
|---------------------------------------------------|----------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.0 stars (a full star under 4.0) | CUSTOMER-SERVICE STAR<br>3 | CLINICAL STAR<br>3.6 | ADDRESSABLE (CS-SLICE)<br>\$10.3M |
| CONTRACTS<br>2                                    | MEMBERS<br>9,841           |                      |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: Baystate's quality bonus dollars under 4.0

Hi Laurie,

There's roughly \$10.3M a year in addressable quality-bonus dollars tied to whether Baystate's Medicare Advantage contracts clear 4.0, and missing by a fraction costs the same as missing by a lot, it's a full cycle of bonus either way. The book is sitting at 3.0 right now, a full star under the line, though your read on the exact figure will be sharper than mine.

The part still in play this cycle is operational, the customer-service and CAHPS measures, since those come down to how the service side runs day to day rather than the clinical side.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures carry Baystate's gap**

---

Hi Laurie,

One more angle on the service-star gap at Baystate. It doesn't sit evenly across the rating, it concentrates in the complaints and customer-service measures, the ones CMS keeps weighting heavier as the ratings evolve. That makes it the piece of the gap that pays back fastest once it moves, since it's tied to how the service operation runs day to day rather than anything clinical.

You'll have a far sharper read than I will on where that shows up most across your book.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what one bad measure cost UHC**

---

Hi Laurie,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure is retired, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book sitting where Baystate's is, that side is most of the ground still available.

A look at which of those measures sit closest to a cut point might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: a measure-by-measure breakdown for Baystate**

---

Laurie, I can put together a measure-by-measure breakdown for Baystate's Medicare Advantage book: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters more than it looks.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks to walk through it?

Nathan Belfield

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**EMAIL 5 Subject: before the December window closes**

---

Hi Laurie,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Baystate's contracts under the bonus line sits in the service measures, and those price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply and I'll send the breakdown. Either way, thanks for the work you're doing here.

Nathan Belfield

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# GuideWell — Dave Petruzzellis

CFO, GOVERNMENT BUSINESS

To: dave.petruzzellis@guidewell.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                  |
|---------------------------------------------------|------------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.8 | CLINICAL STAR<br>3.1 | ADDRESSABLE (CS-SLICE)<br>\$5.8M |
| GROSS FORGONE QBP<br>\$33.4M                      | CONTRACTS<br>2               | MEMBERS<br>62,044    |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: GuideWell's \$5.8M still in play

Hi Dave,

There's an est. \$5.8M a year in quality-bonus dollars still addressable across GuideWell's Medicare Advantage contracts, and missing the 4.0 line by half a star costs the same as missing it by a lot, it's a full cycle of bonus either way. The two contracts are sitting around 3.5 stars right now, though your read on the exact figure will be sharper than mine.

The part still in play this cycle sits in the customer-service measures, which track how the service operation runs day to day rather than anything clinical.

That's what our Dynamic Workforce Orchestration platform does: it shifts and reprioritizes work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where GuideWell's movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: the piece of the gap that pays back**

---

Hi Dave,

One more angle on GuideWell's book. The service-star gap doesn't sit evenly across the two contracts, it concentrates in a handful of customer-service and complaints measures, and those happen to be the ones CMS prices hardest through the December window. That makes this slice the fastest-paying part of the gap to close, even though the clinical side will take longer regardless.

You'll have a far sharper read on which measures matter most for GuideWell's 62k members than I will.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what one bad measure cost UHC**

---

Dave,

Here's why the service side is worth escalating now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That measure's since retired, but the lesson holds: a single customer-service measure priced real money at payer scale.

Teams getting ahead of this aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For GuideWell's book, that side is the part still in play this cycle.

A pressure-test of how you're framing that gap might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: GuideWell's measure-by-measure breakdown**

---

Dave,

I can put together a single page on GuideWell's service-star gap: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters more than it looks.

Humana's freed up about two hours of capacity per agent every month working with us.

I can put together where GuideWell's movable points actually sit and which measures are closest to a cut point. Worth 15 minutes in the next couple weeks to walk it?

Nathan Belfield

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**EMAIL 5 Subject: leaving this here, Dave**

---

Dave,

I'll leave it here so I'm not crowding your inbox. The short version: the addressable dollars in GuideWell's service-star gap price for this cycle only through December, and after that the ground shifts toward CAHPS.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before December, just reply and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# Blue Shield of California — Kassie Maroney

SVP & CHIEF ACTUARY

To: [kassie.maroney@blueshieldca.com](mailto:kassie.maroney@blueshieldca.com) · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.6 | CLINICAL STAR<br>3.2 | ADDRESSABLE (CS-SLICE)<br>\$11.5M |
| GROSS FORGONE QBP<br>\$33.3M                      | CONTRACTS<br>3               | MEMBERS<br>45,627    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: bonus dollars riding on 4.0

Hi Kassie,

There's roughly \$11.5M a year in quality-bonus dollars riding on whether Blue Shield of California's Medicare Advantage book clears 4.0, and missing by half a star costs the same as missing by a lot, it's still a full cycle of bonus gone. The three contracts are sitting at 3.5 right now, though your read on the exact figure will be sharper than mine.

The points still in play this cycle sit in the customer-service measures, which come down to how the service operation runs day to day rather than anything clinical.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

If it helps, I can pressure-test that math against how your team is framing the cliff. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures move the rating**

---

Hi Kassie,

One more angle on the math from my last note. The gap across Blue Shield's three contracts doesn't sit evenly, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. That makes the service side both the biggest lever and the part of a cycle that pays back fastest, though you'll have a far sharper read than I will on where it actually sits for your 45,627 members.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a single measure can cost**

---

Hi Kassie,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has changed since, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book sitting where Blue Shield's is, that service side is most of the gap still worth closing.

Worth 15 minutes to walk the payer-level math against how your team is framing it?

Nathan Belfield

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**EMAIL 4 Subject: the Blue Shield measure breakdown**

---

Kassie, I can put together the measure-by-measure breakdown across Blue Shield's three contracts: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters more than it looks.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk the breakdown? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: before the December window closes**

---

Kassie, I'll leave it here so I'm not crowding your inbox. The short version: real dollars are still riding on the service side across Blue Shield's three contracts, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have. If a look at where those dollars sit is useful before then, just reply here. Either way, appreciate you reading.

Nathan Belfield

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# Imperial Health Plan of California, Inc. — Andranik Barsegyan, MPH

SR. DIRECTOR OF HEALTH ANALYTICS, QUALITY IMPROVEMENT, AND RISK ADJUSTMENT

To: andranik.barsegyan@imperialhealthholdings.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>2.8 | CLINICAL STAR<br>4.2 | ADDRESSABLE (CS-SLICE)<br>\$25.4M |
| GROSS FORGONE QBP<br>\$25.4M                      | CONTRACTS<br>1               | MEMBERS<br>52,035    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Imperial Health Plan's last points sit

Hi Andranik,

For a contract sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data, Imperial Health Plan's customer-service star is sitting well under that line right now. The clinical side is in much better shape, so the service measures look like the clearest place to close the gap, though you'll have a far sharper read than I will. That's roughly \$25.4M a year in addressable bonus tied to those measures.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures move Imperial's rating**

---

Hi Andranik,

One more angle on the gap at Imperial Health Plan. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. So the service side is most of what's holding the book under the line, and it's also the part that pays back fastest once it moves.

You'll have a far sharper read than I will, but it's usually the piece teams underweight relative to clinical work.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi Andranik,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has changed since, but the lesson holds: a single customer-service measure priced real money, and the ones scoring today carry the same weight.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book like Imperial Health Plan's, that side is most of what's still movable this cycle.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: Imperial Health Plan's measure breakdown**

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Hi Andranik,

I can put together the measure-by-measure breakdown for Imperial Health Plan: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters more than usual right now.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk through it? Either way, thanks for the work you're doing here.

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before the December window**

---

Hi Andranik,

I'll leave it here so I'm not crowding your inbox. The short version: the service side is still the movable part of Imperial Health Plan's gap, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# CareFirst BlueCross BlueShield — Ja'Ron Bridges

EXECUTIVE VICE PRESIDENT, CHIEF FINANCIAL OFFICER

To: jaron.bridges@carefirst.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                  |
|---------------------------------------------------|------------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.9 | CLINICAL STAR<br>3.6 | ADDRESSABLE (CS-SLICE)<br>\$5.9M |
| GROSS FORGONE QBP<br>\$22.6M                      | CONTRACTS<br>1               | MEMBERS<br>46,358    |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$5.9M riding on CareFirst's 4.0 line

Hi Ja'Ron,

There's an est. \$5.9M a year in quality-bonus dollars across CareFirst's Medicare Advantage book tied to measures your operations team can still move this cycle, and missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way. The book is sitting at 3.5 right now, though your read on the exact figure will be sharper than mine.

The piece still in play is the customer-service side, since those measures come down to how the service operation runs day to day, and they're scored only through this December.

That's where our Dynamic Workforce Orchestration platform fits: it shifts work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: one more angle on that gap**

---

Hi Ja'Ron,

One more angle on CareFirst's book worth flagging. The gap doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. That makes the service side the part most likely to pay back fastest once it moves, since it's still scored through this December.

You'll have a far sharper read than I will on where your team's already focused.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what one bad measure cost UHC**

---

Ja'Ron,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has changed, but the lesson holds, a single customer-service measure priced real money at that scale.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book sitting where CareFirst's is, that service side is most of what's left to close before the line.

Worth 15 minutes in the next couple weeks to walk the payer-level math?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the CareFirst measure breakdown**

---

Ja'Ron,

I can put together the measure-by-measure breakdown for CareFirst's book: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest. That's more useful than another round of the account-level total.

The complaints and customer-service measures price for this cycle only through December, so the timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk it?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: before CareFirst's December window closes**

---

Ja'Ron,

I'll leave it here so I'm not crowding your inbox. The short version: the movable part of CareFirst's gap sits in the service measures, and those price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before the window closes, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# GuideWell — Jeff Goddard

EVP & CFO GUIDEWELL

To: jeff.goddard@guidewell.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                  |
|---------------------------------------------------|------------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.8 | CLINICAL STAR<br>3.1 | ADDRESSABLE (CS-SLICE)<br>\$5.8M |
| GROSS FORGONE QBP<br>\$33.4M                      | CONTRACTS<br>2               | MEMBERS<br>62,044    |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: GuideWell's bonus dollars riding on 4.0

Hi Jeff,

There's roughly \$5.8M a year in quality-bonus dollars sitting in GuideWell's customer-service measures right now, and that's the piece still movable this cycle. Missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way, and your two Medicare Advantage contracts are sitting right at that half-star gap on the public data, though you'll have a sharper read on it than I will.

The clinical side is its own separate question, so I'll leave that to your team. The service measures come down to how the operation runs day to day, which is the part still in play before the December window.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the window closes. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures move GuideWell's rating**

---

Hi Jeff,

One more angle on the gap we flagged. It doesn't sit evenly across the rating, it concentrates in the complaints and customer-service measures, the ones CMS keeps weighting heavier as Stars evolve. That makes the service side the part of the cycle that pays back fastest across GuideWell's book, covering the 62,000 members on these contracts.

You'll have a far sharper read than I will on which measures are closest to moving.

Worth 15 minutes to walk through which of those sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Jeff, here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has changed since, but the lesson holds: a single customer-service measure priced real money at scale.

The teams getting ahead of this aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book the size of GuideWell's, with two contracts sitting under the bonus line, that service side is most of what's still movable this cycle.

Worth 15 minutes to walk through where that math lands for your contracts?

Nathan Belfield

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**EMAIL 4 Subject: GuideWell's measure-by-measure breakdown**

---

Jeff, I can put together the measure-by-measure breakdown across GuideWell's two contracts: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters more than usual this year.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks to walk through where your points sit?

Nathan Belfield

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**EMAIL 5 Subject: before GuideWell's December window**

---

Jeff, I'll leave it here so I'm not crowding your inbox. The short version: the gap holding GuideWell's contracts under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December. Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have. If it's worth a look before then, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# Point32Health — Michael Marrone

CHIEF FINANCIAL OFFICER

To: michael.marrone@point32health.org · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                  |
|---------------------------------------------------|------------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.2 | CLINICAL STAR<br>3.5 | ADDRESSABLE (CS-SLICE)<br>\$8.3M |
| GROSS FORGONE QBP<br>\$13M                        | CONTRACTS<br>2               | MEMBERS<br>15,506    |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: Point32Health's bonus dollars tied to 4.0

Hi Michael,

There's roughly \$8.3M a year in quality-bonus dollars addressable across Point32Health's two Medicare Advantage contracts, sitting in the customer-service and CAHPS measures your operations team can still move this cycle. Missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way, though your read on the exact figure will be sharper than mine.

Both contracts are sitting around 3.5 stars right now, and the service side is the piece still in play before the December window closes on these measures.

That's where our Dynamic Workforce Orchestration platform fits: it shifts work across queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before that window closes. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Point32's number**

---

Hi Michael,

One more angle on the math from my last note. The gap doesn't sit evenly across the rating, it concentrates in a handful of customer-service and complaints measures, which happen to be the ones an operations team can actually move inside a cycle rather than wait years on.

That's what makes the timing matter. Those measures price for this window through December, so whatever moves, moves now.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what one bad measure can cost**

---

Hi Michael,

Here's why the service side is worth watching closely right now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, its own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure can price out real money.

The operations teams getting ahead of this aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book sitting where Point32Health's contracts are, that side is the part still worth the push this cycle.

Worth 15 minutes to walk the payer-level math together?

Nathan Belfield

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**EMAIL 4 Subject: Point32Health's measure-by-measure breakdown**

---

Michael, I can put together the measure-by-measure breakdown for Point32Health's contracts: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

That's the whole conversation, worth walking through since those measures price for this cycle only through December, so the timing matters.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks to walk it? Either way, thanks for the work you're doing here.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 5 Subject: one last note before December**

---

Hi Michael,

I'll leave it here so I'm not crowding your inbox. The short version: the bonus dollars still in play for Point32Health sit in the service measures, they're recoverable, and they price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before the window closes, just reply and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# Imperial Health Plan of California, Inc. — Meeta Bindra, CPA, MBA

CHIEF FINANCIAL OFFICER

To: meeta.bindra@imperialhealthplan.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>2.8 | CLINICAL STAR<br>4.2 | ADDRESSABLE (CS-SLICE)<br>\$25.4M |
| GROSS FORGONE QBP<br>\$25.4M                      | CONTRACTS<br>1               | MEMBERS<br>52,035    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$25.4M riding on 4.0

Hi Meeta,

There's roughly \$25.4M a year in quality-bonus dollars tied to whether Imperial Health Plan's Medicare Advantage book clears 4.0, and missing by half a star costs the same as missing by a lot, it's a full cycle of bonus either way. The book is sitting at 3.5 right now, though your read on the exact figure will be sharper than mine.

The points still in play this cycle sit in the customer-service measures, which come down to how the service operation runs day to day, and that's the piece an operations team can still move before the window closes in December.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the window closes. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: one more angle on the gap**

---

Hi Meeta,

One more angle worth flagging on Imperial Health Plan's book. The gap doesn't sit evenly across the rating, it concentrates in the customer-service measures, since the clinical side is already well clear of 4.0. That makes the service side effectively the whole lever your team has to pull, which also means every point recovered there drops straight to the bonus line.

You'll have a far sharper read on this than I will, but it's usually the piece finance models miss.

Worth 15 minutes to walk through which measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi Meeta,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure priced real money at scale.

The teams getting ahead of this aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time, which is what makes the gap in Imperial Health Plan's book worth closing sooner rather than later.

A look at which of those measures sit closest to a cut point might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 4 Subject: the Imperial Health Plan measure breakdown**

---

Hi Meeta,

I can put together a measure-by-measure breakdown for Imperial Health Plan's book: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where effort this cycle moves the rating fastest.

That's the whole conversation, 15 minutes to walk it and you decide if there's anything worth acting on. The complaints and customer-service measures only price for this cycle through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

---

Hi Meeta,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Imperial Health Plan's book under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# Clever Care Health Plan — Thomas Manetti

SR DIRECTOR OF RISK ADJUSTMENT AND STARS

To: [thomas.manetti@ccmapd.com](mailto:thomas.manetti@ccmapd.com) · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                            |                      |                                   |
|---------------------------------------------------|----------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3 | CLINICAL STAR<br>3.8 | ADDRESSABLE (CS-SLICE)<br>\$21.2M |
| GROSS FORGONE QBP<br>\$24.5M                      | CONTRACTS<br>1             | MEMBERS<br>50,156    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Clever Care's last points sit

Hi Thomas,

For a contract sitting half a star under the 4.0 line, the last points almost always concentrate in the service side, the customer-service and CAHPS experience measures. Those track how the operation runs day to day, so they're the part still in play this cycle.

On the public data, the customer-service star is sitting at 3 right now, a notch below where the clinical side lands. That works out to about \$21.2M a year in addressable bonus tied to service operations, though you'll have a far sharper read on that than I will.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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**EMAIL 2 Subject: which measures move Clever Care's rating**

---

Thomas, one more angle on Clever Care's gap. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier into 2029. So the service side carries most of what's holding the book under the bonus line, which also makes it the part that pays back fastest once it moves. You'll have a far sharper read than I will, but it's usually the piece teams underweight. Worth 15 minutes to walk which of those measures sit closest to a cut point?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi Thomas,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since retired, but the lesson holds: a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time, which is what our Dynamic Workforce Orchestration platform does. For a book sitting where Clever Care's is, that service side is most of the gap left to close.

Worth 15 minutes in the next couple weeks to walk the payer-level math?

Nathan Belfield

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**EMAIL 4 Subject: the Clever Care measure breakdown**

---

Hi Thomas,

I can put the measure-by-measure breakdown for Clever Care's book on a single page: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here. Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk it? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

---

Hi Thomas,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Clever Care's book under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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# Blue Shield of California — SarahKay Hermosillo

SR. DIRECTOR – BUSINESS TECHNOLOGY & AI ENABLEMENT – MEMBER EXPERIENCE & HEALTH PLAN CORRESPONDENCE

To: sarahkay.hermosillo@blueshieldca.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.6 | CLINICAL STAR<br>3.2 | ADDRESSABLE (CS-SLICE)<br>\$11.5M |
| GROSS FORGONE QBP<br>\$33.3M                      | CONTRACTS<br>3               | MEMBERS<br>45,627    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Blue Shield's last points sit

Hi SarahKay,

For plans sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data, Blue Shield of California's Medicare Advantage contracts sit right around 3.5, and the customer-service star is at 3.6, just above that. That puts an est. \$11.5M a year in addressable bonus across your book in reach, though you'll have a far sharper read than I will.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures carry the gap**

---

Hi SarahKay,

One more angle on Blue Shield's gap. It doesn't spread evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier looking toward 2029. That makes the service side most of what's holding the book under the bonus line, and also the part that pays back fastest once it moves.

You'll have a far sharper read than I will, but it's usually the piece teams underweight against the clinical side.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a service measure is worth**

---

Hi SarahKay,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure has since retired, but the lesson holds: a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time. For a book sitting where Blue Shield's is, that service side is most of the gap to the bonus line.

Worth 15 minutes in the next couple weeks to walk the payer-level math?

Nathan Belfield

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**EMAIL 4 Subject: the Blue Shield measure breakdown**

---

Hi SarahKay,

I can put the measure-by-measure breakdown for Blue Shield's book on a single page: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so the timing matters more than it looks.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk it? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

---

Hi SarahKay,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Blue Shield's book under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the service-measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Point32Health — Laure Goldberg

SVP AND CHIEF ACTUARY

To: laure.goldberg@point32health.org · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                  |
|---------------------------------------------------|------------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.2 | CLINICAL STAR<br>3.5 | ADDRESSABLE (CS-SLICE)<br>\$8.3M |
| GROSS FORGONE QBP<br>\$13M                        | CONTRACTS<br>2               | MEMBERS<br>15,506    |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the \$8.3M riding on Point32's line

Hi Laure,

There's roughly \$8.3M a year in quality-bonus dollars addressable across Point32Health's Medicare Advantage contracts, and missing the 4.0 line by half a star costs the same as missing by a lot, it's a full cycle of bonus either way. The contracts are sitting around 3.5 right now, so that half star is what's deciding the outcome, though your read on the exact figure will be sharper than mine.

The part still in play this cycle is the customer-service side, since those measures come down to how the service operation runs day to day, and they're scored for the last time this way before December.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the window closes. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures carry Point32's gap**

---

Hi Laure,

One more angle on the gap across Point32Health's contracts. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier into 2029. That makes the service side the part most likely to move fastest once an operations team focuses there, since it tracks day-to-day handling rather than anything clinical.

You'll have a far sharper read than I will on which specific measures are closest to tipping.

Worth 15 minutes to walk through which ones sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Laure,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, its own figure, under the prior rules. That specific measure has since retired, but the lesson holds: a single customer-service measure priced real money. The teams getting ahead of it aren't adding headcount, they're using tools like our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book sitting where Point32Health's contracts are, that service side is most of the gap to the bonus line.

Worth 15 minutes to walk through where your movable points sit closest to a cut point?

Nathan Belfield

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**EMAIL 4 Subject: the Point32Health measure breakdown**

---

Hi Laure,

I can put together the measure-by-measure breakdown across Point32Health's contracts: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the needle fastest. That's the whole conversation, 15 minutes to walk it and you decide if there's anything worth acting on. The customer-service and complaints measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks?

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

---

Hi Laure,

I'll leave it here so I'm not crowding your inbox. The short version: a good chunk of the gap holding Point32Health's contracts under the bonus line sits in the customer-service measures, and those price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Clever Care Health Plan — Richard Greene

CHIEF FINANCIAL OFFICER

To: richard.greene@ccmapd.com · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                            |                      |                                   |
|---------------------------------------------------|----------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3 | CLINICAL STAR<br>3.8 | ADDRESSABLE (CS-SLICE)<br>\$21.2M |
| GROSS FORGONE QBP<br>\$24.5M                      | CONTRACTS<br>1             | MEMBERS<br>50,156    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the dollars riding on 4.0

Hi Richard,

There's an est. \$21.2M a year in quality-bonus dollars addressable across Clever Care's Medicare Advantage book, and missing the 4.0 line by half a star costs the same as missing it by a lot, it's a full cycle of bonus gone either way. The book is sitting around 3.5 stars right now, though your read on the exact figure will be sharper than mine.

The part still in play this cycle is the service side, the customer-service and CAHPS measures, since those come down to how the operation runs day to day.

That's where our Dynamic Workforce Orchestration platform fits: it reprioritizes work across queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures carry Clever Care's gap**

---

Hi Richard,

One more angle on the math from my last note. The gap doesn't sit evenly across Clever Care's book, it concentrates in a handful of customer-service and complaints measures, and those are the ones still open to move before the December cutoff.

That also makes them the fastest-paying part of the fix, since every point recovered there counts toward the same bonus cycle. You'll have a far sharper read than I will on which ones are closest.

Worth 15 minutes to walk through which measures sit closest to a cut point?

Nathan Belfield

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2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 3 Subject: what a single measure can cost**

---

Hi Richard,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That measure's since retired, but the lesson holds: a single customer-service measure priced real money.

The finance teams getting ahead of this aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book sitting where Clever Care's is, that service side is most of what stands between you and the bonus line.

A pressure-test of that math might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: the payer-level measure breakdown**

---

Richard, I can put together the measure-by-measure breakdown for Clever Care: which customer-service and CAHPS measures sit closest to a cut point, and where a cycle's effort moves the rating fastest across your book.

The complaints and customer-service measures price for this cycle only through December, so the timing matters more than it looks.

Humana's freed up about two hours of capacity per agent every month working with us, which is the kind of operational lift that shows up in these measures.

Worth putting time on the calendar in the next couple weeks to walk the breakdown? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: before Clever Care's December window**

---

Richard, I'll leave it here so I'm not crowding your inbox. The short version: the bonus dollars tied to Clever Care's service-star gap are still recoverable, and those measures price for this cycle only through December. Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have. If it's worth a look before the window closes, just reply and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Community Health Plan of Washington — Marie Faulring

VICE PRESIDENT OF FINANCE

To: marie.faulring@chpw.org · Finance / Economic buyer · Persona 2

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                            |                      |                                  |
|---------------------------------------------------|----------------------------|----------------------|----------------------------------|
| OVERALL BOOK<br>3.0 stars (a full star under 4.0) | CUSTOMER-SERVICE STAR<br>3 | CLINICAL STAR<br>3.1 | ADDRESSABLE (CS-SLICE)<br>\$4.9M |
| CONTRACTS<br>1                                    | MEMBERS<br>8,975           |                      |                                  |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: the dollars sitting under 4.0

Hi Marie,

There's roughly \$4.9M a year in quality-bonus dollars addressable across Community Health Plan of Washington's Medicare Advantage book, tied to the customer-service and CAHPS measures, and missing the 4.0 line by a fraction costs the same as missing by a lot. It's a full cycle of bonus either way. The book is sitting at 3.0 right now, a full star under the line, though your read on the exact math will be sharper than mine.

Realistically that puts you a full star out, so this reads as a longer build rather than a same-cycle fix, and where you start now shapes how fast that gap closes.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold on the days that usually slip. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: one more angle on that gap**

---

Hi Marie,

One more angle on Community Health Plan of Washington's book. The gap doesn't sit evenly across the rating, it concentrates in customer-service, complaints, and appeals, the measures CMS keeps weighting heavier through 2029. So that's the slice that's both movable and the fastest to pay back, though your read on where it sits will beat mine.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a single measure can cost**

---

Hi Marie,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure has since been retired, but the lesson holds, a single customer-service measure priced real money.

Teams getting ahead of this aren't adding headcount, they're using tools like ours to move several service and CAHPS measures at once in real time, which is what keeps a book like yours from staying parked below the bonus line.

A look at which of those measures sit closest to a cut point might be useful. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 4 Subject: the measure-level breakdown for your book**

---

Marie, I can put together a measure-by-measure breakdown for Community Health Plan of Washington: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk through where your movable points sit? Either way, thanks for the work you're doing here.

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

---

Hi Marie,

I'll leave it here so I'm not crowding your inbox. The short version: the dollars riding on Community Health Plan of Washington's book concentrate in the customer-service and CAHPS measures, and those price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Medica — Bobbie McAdam

VP, MARKET OPERATIONS AND EFFECTIVENESS

To: [bobbie.mcadam@medica.com](mailto:bobbie.mcadam@medica.com) · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.8 | CLINICAL STAR<br>3.6 | ADDRESSABLE (CS-SLICE)<br>\$23.7M |
| GROSS FORGONE QBP<br>\$61.2M                      | CONTRACTS<br>3               | MEMBERS<br>112,248   |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Medica's last points sit

Hi Bobbie,

For plans sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data, Medica's three Medicare Advantage contracts sit right around 3.5, with the customer-service star at 3.8 a notch ahead of the clinical side. That works out to roughly \$23.7M a year in addressable bonus across the book, though you'll have a far sharper read than I will.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Medica's rating**

---

Hi Bobbie,

One more angle on Medica's gap. It doesn't sit evenly across the rating, it concentrates in the complaints and customer-service measures, the ones CMS keeps weighting heavier as Stars evolve. That makes the service side the part of the cycle that pays back fastest once it moves, since it's operational and within an ops team's reach this year.

You'll have a far sharper read than I will on where those measures actually sit across your three contracts.

Worth 15 minutes to walk through which of them sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Bobbie,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since changed, but the lesson holds, a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using Intradiem to move several of the service and CAHPS measures at once in real time. For a book the size of Medica's, that service side is most of the gap standing between your contracts and the bonus line.

Worth 15 minutes in the next couple weeks to walk the payer-level math?

Nathan Belfield

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**EMAIL 4 Subject: Medica's measure-by-measure breakdown**

---

Bobbie, I can put together a payer-level breakdown for Medica's three contracts: which customer-service and CAHPS measures sit closest to a cut point, how much of the addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures only price for this cycle through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting time on the calendar in the next couple weeks to walk through it?

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

---

Hi Bobbie,

I'll leave it here so I'm not crowding your inbox. The short version: the movable part of Medica's gap sits in the service measures, and those price for this cycle only through December.

If it's worth a look before then, just reply and I'll send the breakdown. Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

Either way, appreciate you reading.

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

# Centene — Matthew Tran

SR DIRECTOR, MEMBER EXPERIENCE AND INNOVATION

To: matthew.tran@centene.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>2.8 | CLINICAL STAR<br>2.7 | ADDRESSABLE (CS-SLICE)<br>\$52.5M |
| GROSS FORGONE QBP<br>\$165.8M                     | CONTRACTS<br>21              | MEMBERS<br>60,651    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Centene's last points sit

Hi Matthew,

For books sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data your Medicare Advantage contracts sit right around 3.5 across the book, with the customer-service star lagging the clinical side. That's roughly \$52.5M a year in addressable bonus tied to those service measures, though you'll have a far sharper read than I will on how it breaks out.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

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**EMAIL 2 Subject: which measures move Centene's rating**

---

Hi Matthew,

One more angle on the gap across Centene's book. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. That makes the service side most of what's holding the 21 contracts under the line, and also the part of a cycle that pays back fastest once it moves.

You'll have a far sharper read than I will on which measures are closest to tipping.

Worth 15 minutes to walk through which of those sit nearest a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

---

Hi Matthew,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has since retired, but the lesson holds: a single customer-service measure can price real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. Across Centene's 21 contracts sitting under the line, that service side is most of the gap worth closing.

Worth 15 minutes to walk through the payer-level math on which measures move first?

Nathan Belfield

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**EMAIL 4 Subject: Centene's measure-by-measure breakdown**

---

Hi Matthew,

I can put together the payer-level breakdown across Centene's 21 contracts under the line: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest. That's a sharper starting point than the account-wide total we opened with.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth 15 minutes in the next couple weeks to walk through it?

Nathan Belfield

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**EMAIL 5 Subject: before the December window, Matthew**

---

Hi Matthew,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Centene's book under the line sits in the service measures, they're recoverable, and they price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the breakdown.

Nathan Belfield

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# Medica — Kim Bachmeier

SR DIRECTOR, CUSTOMER SERVICE STRATEGY & OPTIMIZATION

To: kim.bachmeier@medica.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>3.8 | CLINICAL STAR<br>3.6 | ADDRESSABLE (CS-SLICE)<br>\$23.7M |
| GROSS FORGONE QBP<br>\$61.2M                      | CONTRACTS<br>3               | MEMBERS<br>112,248   |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Medica's last points sit

Hi Kim,

For plans sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data your Medicare Advantage contracts sit right around 3.5 across the book, with the customer-service star at 3.8. Roughly \$23.7M of the total forgone bonus sits in those service measures, though you'll have a far sharper read than I will on how that splits across your contracts.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

Pipeline Generation Lead | [www.intradiem.com](http://www.intradiem.com)

937.238.3179

2500 Dallas Hwy | Ste 202 | DPT#37049 | Marietta, GA 30064

**EMAIL 2 Subject: which measures move Medica's rating**

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Hi Kim,

One more angle on the gap across Medica's contracts. It doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. So the service side is most of what's holding the book under the line, which also makes it the part that pays back fastest once it moves.

You'll have a far sharper read than I will, but it's usually the piece teams underweight.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 3 Subject: what a service measure is worth**

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Hi Kim,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. That specific measure has since been retired, but the lesson still holds: a single customer-service measure priced real money.

The teams getting ahead of this aren't adding headcount, they're using Intradiem to move several service and CAHPS measures at once in real time, which is what actually shifts a rating sitting where Medica's book is.

Worth 15 minutes to walk through which of those measures sit closest to a cut point?

Nathan Belfield

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**EMAIL 4 Subject: the Medica measure breakdown**

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Hi Kim,

I can put the measure-by-measure breakdown for Medica's book on a single page: which customer-service and CAHPS measures sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so the timing matters more than it looks like on paper.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk through it?

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

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Hi Kim,

I'll leave it here so I'm not crowding your inbox. The short version: the gap holding Medica's book under the bonus line sits in the service measures, they're recoverable, and those measures price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before then, just reply here and I'll send the measure breakdown. Either way, appreciate you reading.

Nathan Belfield

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# Centene — Cyrus Hoseini

VP, OPERATIONS

To: cyrus.hoseini@centene.com · Stars / Quality · Persona 1

DEFEND THE NUMBERS · PUBLIC CMS 2026 STAR RATINGS (OCT 2025 RELEASE)

|                                                   |                              |                      |                                   |
|---------------------------------------------------|------------------------------|----------------------|-----------------------------------|
| OVERALL BOOK<br>3.5 stars (half a star under 4.0) | CUSTOMER-SERVICE STAR<br>2.8 | CLINICAL STAR<br>2.7 | ADDRESSABLE (CS-SLICE)<br>\$52.5M |
| GROSS FORGONE QBP<br>\$165.8M                     | CONTRACTS<br>21              | MEMBERS<br>60,651    |                                   |

The service side (customer-service + CAHPS) is the movable part this cycle; those measures score only through December. Clinical is separate.

EMAIL 1 Subject: where Centene's last points sit

Hi Cyrus,

For plans sitting half a star under the 4.0 line, the last few points almost always come from the service side, the customer-service and CAHPS experience scores. Those track how the operation runs day to day, so they're the movable part, and the piece still in play this cycle.

On the public data Centene's book sits right around 3.5 across 21 contracts, with the customer-service star running under the clinical side. About \$52.5M of the addressable bonus sits in those service measures, though you'll have a far sharper read than I will on how that breaks out.

That's what our Dynamic Workforce Orchestration platform does: it shifts work across your queues in real time so service levels hold when volume spikes. Humana cut average handle time by 45 seconds working with us, operationally, and I'm careful not to read that as a Stars number.

A quick read on where your movable points actually sit might be useful before the December window. Worth 15 minutes in the next couple weeks?

Nathan Belfield

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**EMAIL 2 Subject: which measures move Centene's rating**

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Hi Cyrus,

One more angle on Centene's book. The gap doesn't sit evenly across the rating, it concentrates in the customer-service and complaints measures, the ones CMS keeps weighting heavier through 2029. That makes the service side most of what's holding those 21 contracts under the line, and also the piece that pays back fastest once it moves.

You'll have a far sharper read than I will on which measures are closest to a cut point across 60,651 members.

Worth 15 minutes to walk through which ones are worth tackling first?

Nathan Belfield

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**EMAIL 3 Subject: what a customer-service measure is worth**

---

Hi Cyrus,

Here's why the service side is worth the attention now. UnitedHealthcare told a federal court that one failed call-center secret-shopper call cost it \$190M in Star bonus payments, their own figure, under the prior rules. The specific measure has changed, but the lesson holds: a single customer-service measure priced real money.

The teams getting ahead of it aren't adding headcount, they're using our Dynamic Workforce Orchestration platform to move several service and CAHPS measures at once in real time. For a book the size of Centene's, that's most of what stands between those 21 contracts and the bonus line.

A pressure-test of which measures move fastest for your book might be worth it. Got 15 minutes in the next couple weeks?

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**EMAIL 4 Subject: the Centene measure breakdown**

---

Cyrus, I can put the measure-by-measure breakdown for Centene's book on a single page: which customer-service and CAHPS measures across those 21 contracts sit closest to a cut point, how much addressable bonus each one carries, and where a cycle's effort moves the rating fastest.

The complaints and customer-service measures price for this cycle only through December, so timing matters here.

Humana's freed up about two hours of capacity per agent every month working with us.

Worth putting 15 minutes on the calendar in the next couple weeks to walk it?

Nathan Belfield

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**EMAIL 5 Subject: before the December window**

---

Hi Cyrus,

I'll leave it here so I'm not crowding your inbox. The short version: the movable part of Centene's gap sits in the service measures across those 21 contracts, and they price for this cycle only through December.

Humana's own team calls our dynamic workforce orchestration system table stakes now, not a nice-to-have.

If it's worth a look before the window closes, just reply and I'll send the breakdown. Either way, appreciate you reading.

Nathan Belfield

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